DELIVERED PRIVATELY TO

Dr. Chima Oti, Managing Director, and the Board
Dennis Ashley Medical Clinic & Endoscopy Suites

25 September 2026
Ref: MZ-ENG-DAMC-2026-BU

REVENUE BUILD-UP

What the building can earn: built from rooms, sessions and lists, not from percentages

This is the growth case behind the engagement proposal, built from the bottom up. We have started from the clinic exactly as it stands: **two consulting rooms, one treatment room, an endoscopy suite with a two-bed recovery, a mini pharmacy and an outsourced laboratory**. No new equipment, no building works, no capital from either side. Every naira below traces back to a session sold, a slot filled or a list run, and prices are taken from our live catalogue at realised rates, blended across cash and payer.

> **This is a jumpstart, not an optimisation.** We have modelled the clinic as it appears to be trading today, which on our estimate is **around N2.5m a month**, and we hold that flat for the whole twelve months. We do not claim credit for it and we do not assume it grows on its own. That means **84% of year-one revenue in this plan is revenue that does not exist today**, and every naira of it is ours to produce. If the true figure is higher than our estimate, the plan simply starts from a better place: the build above does not change.

MONTH 12 RUN RATE
N30.9m
a month, against roughly N2.5m today

YEAR 1 REVENUE
N201.2m
against N30m if nothing changes

MONTH 12 CONTRIBUTION
N14.5m
a month, before fixed costs

NEW REVENUE, YEAR 1
84%
of the total does not exist today

1 · What the building can hold

Asset	Weekly ceiling	What bounds it
2 consulting rooms	24 sessions, up to 12 patients a session	Two four-hour sessions a day across six days. A clinic session is not slot-bound the way a theatre list is: patients queue in reception, so twenty minutes a patient is the working assumption and the room, not the diary, is the constraint.
Endoscopy suite 2-bed recovery	3 lists, up to 8 cases a list	Recovery is the true constraint, not the scope, and it is the one thing that cannot be queued: a sedated patient occupies a bed for an hour whatever the waiting room looks like. A four-hour list against two beds caps out at eight.
Treatment room	Continuous	Injections, infusions, dressings, minor and regenerative procedures. Runs alongside clinics rather than competing with them.
Mini pharmacy	Every script written	Bounded by capture rate, not by space. The question is what share of scripts written in the building are dispensed in the building.

Asset	Weekly ceiling	What bounds it
Outsourced laboratory	Unbounded	No capacity limit at all. Bounded only by how many consultations generate a request, and by the share of the billing retained after the laboratory is paid.

2 · The price card, and the payer mix behind it

Two decisions drive every naira in this plan, and they are the reason revenue per patient is high while patient numbers stay modest.

- **A health check is sold whole and delivered in parts.** The clinic performs the consultation, the phlebotomy and the endoscopy in house; laboratory analysis and imaging happen elsewhere, the imaging at a partner centre minutes away. The patient buys one package, at one price, from one front desk, and never manages the handoffs. That is the product. It also means roughly half of a check's value is bought in at network rates rather than earned on site, which the contribution line reflects.
- **Specialist care, not primary care.** The rooms are filled by orthopaedic surgeons, women's health, bariatric and general surgery, metabolic medicine and paediatrics, not by general practice. On our catalogue that is a cash consultation mix averaging **N56,660**. The generic N13,200 GP line does not apply here: your own general practitioners are UK-trained and price accordingly, and the entry tier is a medical officer at N20,000.
- **80% cash to 20% payer.** Payer tariffs realise around half of cash, so the mix itself is worth roughly **90% of the cash card**. We are deliberately building a cash-weighted book and keeping the credit business small.

Line	Used here	How it is derived
Consultation, blended	N48,000	Specialist-weighted cash mix of N56,660 (medical officer N20,000, UK-trained GP N40,000, specialist N52,800, orthopaedic N66,000, visiting consultant N100,000), at the 90% payer blend, less 6% for discounting and follow-ups.
Endoscopy case, blended	N225,000	Gastroscopy N246,400 to N254,800 cash and colonoscopy N264,000, at the payer blend.
Treatment room procedure	N63,000	Specialist procedures, image-guided injections and regenerative work, not dressings and observations.
Dispensed script	N15,000	Specialist prescribing basket, costed at a 35% retained margin.
Laboratory panel	N31,000	Specialist workup rather than a single test: E/U/Cr N19,600, HbA1c N21,000, lipid profile N23,520. 60% paid away to the laboratory.
Executive health check	N475,000	Packaged premium screening, sold for cash and in tiers: standard N250,000, comprehensive N500,000, platinum and international plans reaching N1,000,000. The blend assumes half the volume sits on the standard tier. Roughly 65% of the check is delivered outside the building (laboratory analysis and imaging), bought at network rates and carried at a 53% cost of delivery in the contribution line.

> **Less credit, more cash.** A book that is 80% cash collects at the point of care instead of ninety days later. That matters three ways: the clinic funds its own growth out of its own cash rather than out of receivables, the eleven-payer panel stops setting the price of everything, and our management fee, which is paid on *collected* revenue, only earns when the money has actually landed. Payer work stays in the book where it fills a quiet session or brings a corporate account with it, and not otherwise.

3 · The twelve months, line by line

Month	Consults	Scopes	Existing	Consults	Endoscopy	Treatment	Pharmacy	Lab	Other	TOTAL	Contribution
M1	0	0	2,500,000	0	0	0	0	0	0	**2,500,000**	875,000
M2	17	3	2,550,000	831,360	730,688	272,790	227,940	272,487	0	**4,885,264**	2,069,387
M3	39	3	2,600,000	1,870,560	730,688	545,580	457,056	511,416	0	**6,715,300**	2,960,923
M4	65	3	2,650,000	3,117,600	730,688	818,370	721,245	825,514	575,000	**9,438,417**	4,270,785

Month	Consults	Scopes	Existing	Consults	Endoscopy	Treatment	Pharmacy	Lab	Other	TOTAL	Contribution
M5	95	6	2,700,000	4,572,480	1,461,375	1,091,160	1,024,404	1,323,508	700,000	**12,872,927**	5,952,212
M6	119	8	2,725,000	5,715,600	1,704,938	1,091,160	1,308,728	1,711,432	1,350,000	**15,606,858**	7,231,770
M7	156	8	2,750,000	7,482,240	1,704,938	1,363,950	1,685,280	2,264,460	1,525,000	**18,775,868**	8,731,769
M8	156	9	2,775,000	7,482,240	1,948,500	1,363,950	1,798,662	2,346,340	2,175,000	**19,889,692**	9,257,520
M9	197	13	2,800,000	9,456,720	2,922,750	1,636,740	2,171,195	3,089,975	2,350,000	**24,427,379**	11,461,287
M10	197	15	2,800,000	9,456,720	3,288,094	1,909,530	2,290,747	3,201,386	2,950,000	**25,896,476**	12,189,061
M11	242	15	2,800,000	11,639,040	3,288,094	1,909,530	2,710,368	3,910,791	3,075,000	**29,332,823**	13,769,600
M12	242	16	2,800,000	11,639,040	3,653,438	2,182,320	2,836,740	4,111,465	3,675,000	**30,898,002**	14,535,466
Year 1	**1526**	**99**	**32,450,000**	**73,263,600**	**22,164,188**	**14,185,080**	**17,232,364**	**23,568,775**	**18,375,000**	**201,239,006**	**93,304,782**

Volumes are incremental. “Existing” carries the current trading level plus a capture uplift that reaches 12% by month nine, from billing every encounter through the platform, reconciling collections daily and chasing claims across the eleven-payer panel to settlement. “Other” is executive health, home visits and new corporate contracts. Contribution is after professional fees to the bench, pharmacy cost of goods, the laboratory pass-through, endoscopy consumables and the monthly marketing budget, and before rent, payroll and utilities.

4 · What this actually asks of the clinic

The figures are large only because Lagos specialist prices are large. The *activity* behind them is modest, and that is the point:

- **9 extra patients a working day** by month twelve, across two rooms and six visiting practices. Roughly three per room.
- **4 endoscopy cases a week** by month twelve, against a suite that could take twenty-four. Bariatric assessment alone should carry most of this.
- **42% of sellable room sessions** in use at month twelve, and 58% of the slots within them. Half the building is still idle at the end of year one.

> This is why we say what we say about space. The plan above does not need another square metre: **it fits inside the clinic as it stands today**. But on this trajectory the rooms are the binding constraint somewhere in year two, not year one. When we come to the board asking for more space, it will be with these utilisation figures in hand, and not before.

5 · If our estimate of the baseline is wrong

The build is independent of where the clinic starts. Only the flat line moves, so the board can read the plan against whatever the real figure turns out to be:

If the clinic trades at	Year 1, doing nothing	Year 1, this plan	Month 12 run rate	Share that is new
N1.0m a month	N12m	N181.8m	N29.2m	93%
N2.5m a month	N30m	N201.2m	N30.9m	84%
N4.0m a month	N48m	N220.7m	N32.6m	76%

Incremental revenue in year one is N168.8m in every case. That is the number we are accountable for, and it is the number the performance side of our fee should be measured on.

6 · What we have deliberately not assumed

- **No price rises.** The card above is held flat for twelve months. Any tariff renegotiation across the eleven payers is upside we have not counted.
- **No improvement in the payer mix.** The 80% cash share is held flat throughout, although a specialist book normally becomes more cash-weighted as it matures, not less.
- **No capital.** No new equipment, no additional rooms, no building works, from either party.
- **No growth in the existing book.** Whatever the clinic trades at today is held flat for twelve months, which is conservative for a practice that is about to be a great deal busier.

- **No inpatient or theatre revenue.** Operative work goes to network theatres. Dennis Ashley earns the consultation, the workup, the diagnostics and the follow-up, which is the day-case economics, not the surgical fee.
- **A full marketing line from month one.** N750,000 a month is carried as a cost from the first month, before it has produced anything.

7 · What would break it

- **Sessions that are sold but not filled.** The risk is not finding specialists, it is specialists sitting in a room with four patients booked. Fixed by central booking, published prices and a referral protocol, all of which we run.
- **The suite stalling.** Endoscopy is roughly a fifth of month-twelve revenue. If bariatric and metabolic workup does not convert into scopes, the shortfall is real and visible early.
- **Collections rather than billings.** Our fee is paid on cash, so payer ageing hurts us before it hurts the board. That is deliberate.
- **The marketing line going unfunded.** The build assumes the budget is committed and spent on schedule. A stop-start budget produces a stop-start ramp.

8 · What replaces the guesswork

Two numbers in this model are ours to prove and one is yours to supply. The volumes and the price card are ours. The **baseline and the operating cost lines are yours**, and once we have twelve months of revenue, collections and management accounts, this model stops being an illustration and becomes the plan we are measured against. Nothing in the engagement waits on that, but the sooner it arrives, the sooner the targets in the Management Agreement are written against your numbers rather than our estimates.

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